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Time taken	17 secs
Marks	0/27
Grade	0 out of 100



Question 1

Not answered

Marked out of 1

An economic analysis was conducted to determine the relative cost-effectiveness of three classes of antidepressants: tricyclic antidepressants (TCAs), selective serotonin reuptake inhibitors (SSRIs), and the modified TCA Lofepamine (LOF) as first choice treatments for depression in primary care. [Peveler, R., et al. (2005). A randomised controlled trial to compare the cost-effectiveness of tricyclic antidepressants, selective serotonin reuptake inhibitors and Lofepamine]. Using the below graph answer the following questions. In this context, the acronym QALY is most likely to indicate



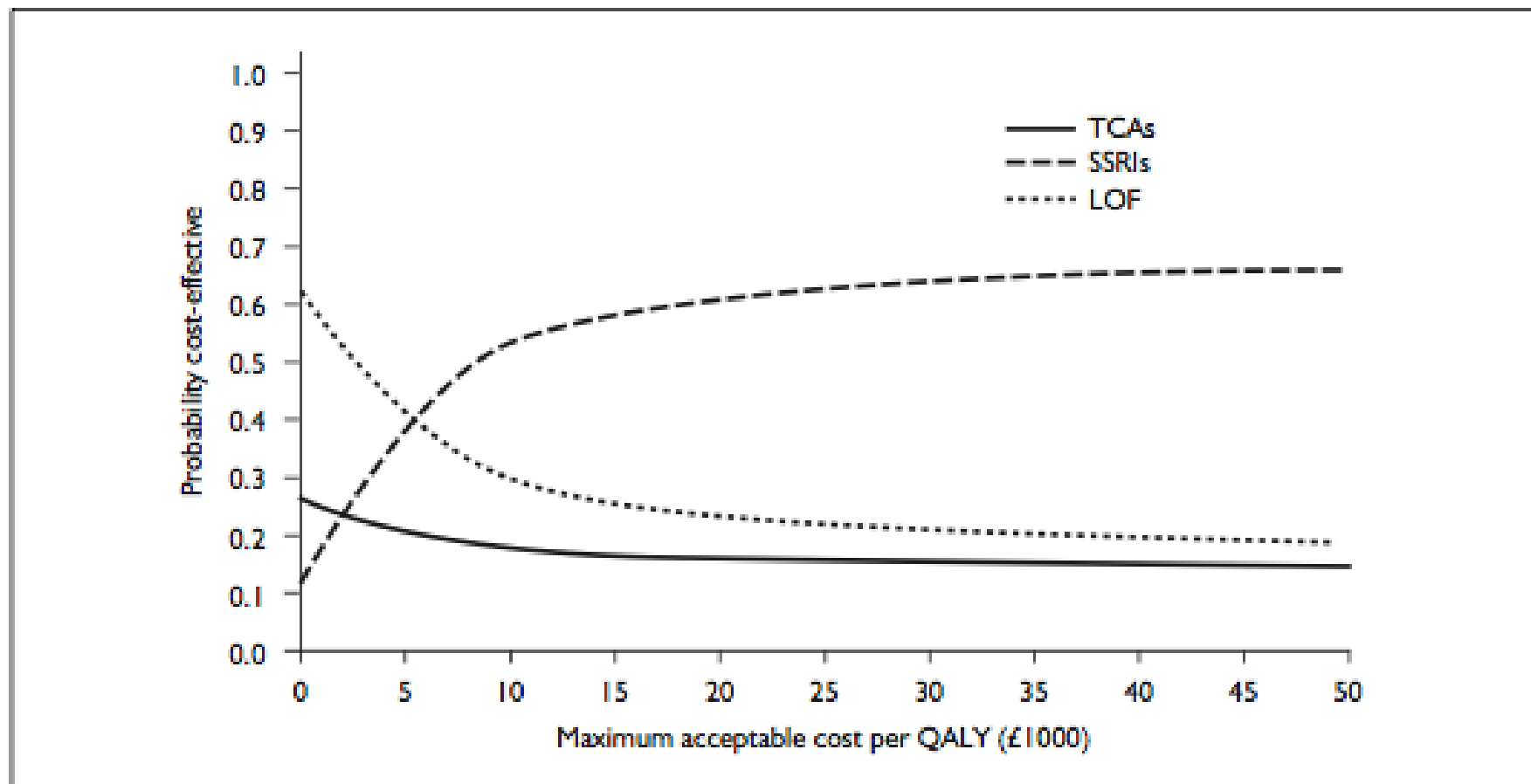


FIGURE 17 CEAC: cost per QALY

Select one:

- ☐ Quality Approximated Life Years
- ☐ Quantity of Actual Life Years

- ☐ Quantity Acceptable for Lending Years
- ☐ Quality Adjusted Lender's Yield
- ☐ Quality Adjusted Life Years

Your answer is incorrect.

QALY refers to quality-adjusted life-year. It is obtained by computing the mathematical product of life expectancy and a measure of the quality of the unspent (remaining) life-years. Rapley, M. (2003). Quality of life research: A critical introduction. Sage.

The correct answer is: Quality Adjusted Life Years



Question 2

Not answered

Marked out of 1

An economic analysis was conducted to determine the relative cost-effectiveness of three classes of antidepressants: tricyclic antidepressants (TCAs), selective serotonin reuptake inhibitors (SSRIs), and the modified TCA Lofepramine (LOF) as first choice treatments for depression in primary care. [Peveler, R., et al. (2005). A randomised controlled trial to compare the cost-effectiveness of tricyclic antidepressants, selective serotonin reuptake inhibitors and Lofepramine]. From the below graph answer the following questions. If a sum of £50,000 is invested on the provision of TCAs, then the probability that this investment has contributed to a gain in 1 QALY is



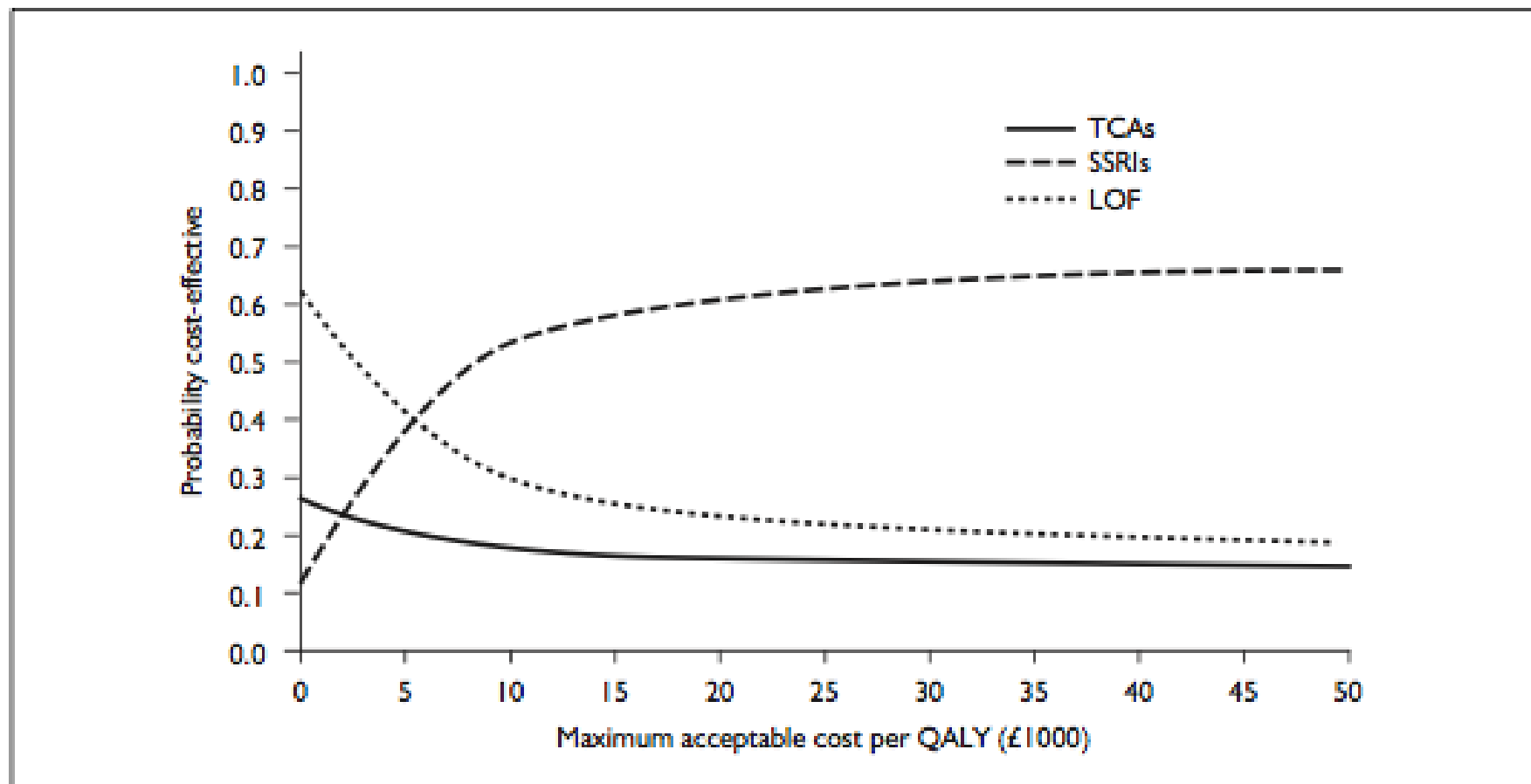


FIGURE 17 CEAC: cost per QALY

Select one:

- ☐ 20%
- ☐ 0.7

- ☐ 30%
- ☐ 0.2%
- ☐ 60%

Your answer is incorrect.

In the attached graph, the continuous line refers to TCAs. Along the x axis, we can see that £50,000 per QALY equates to a point of 0.2 along the y axis for the TCA curve. The value 0.2 is same as a 20% probability for achieving an acceptable cost-effectiveness at the cost specified.

The correct answer is: 20%



Question **3**

Not answered

Marked out of 1

An economic analysis was conducted to determine the relative cost-effectiveness of three classes of antidepressants: tricyclic antidepressants (TCAs), selective serotonin reuptake inhibitors (SSRIs), and the modified TCA Lofepramine (LOF) as first choice treatments for depression in primary care. [Peveler, R., et al. (2005). A randomised controlled trial to compare the cost-effectiveness of tricyclic antidepressants, selective serotonin reuptake inhibitors and Lofepramine]. From the below graph answer the following questions. The attached graph is known as



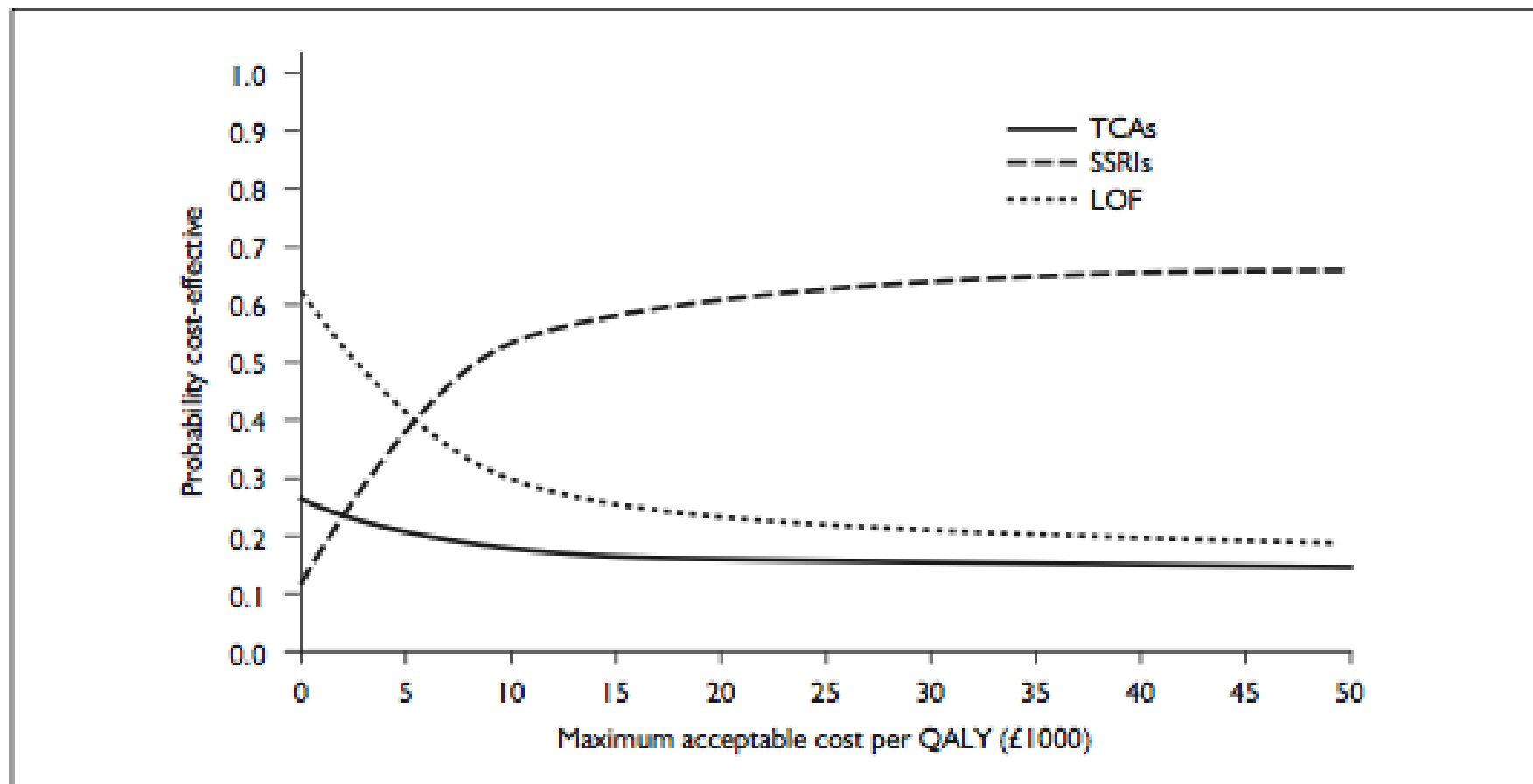


FIGURE 17 CEAC: cost per QALY

Select one:

- ☐ Cost Effectiveness Plane
- ☐ Kaplan Meier Curve

- ☐ Cost Effectiveness Acceptability Curve
- ☐ Cost Effectiveness Analysis Curve
- ☐ ICER graph

Your answer is incorrect.

A cost-effectiveness acceptability curve (CEAC) is a method to represent uncertainty associated with economics of health interventions. It is frequently used to present the results of a cost-effectiveness analysis

The correct answer is: Cost Effectiveness Acceptability Curve



Question 4

Not answered

Marked out of 1

An economic analysis was conducted to determine the relative cost-effectiveness of three classes of antidepressants: tricyclic antidepressants (TCAs), selective serotonin reuptake inhibitors (SSRIs), and the modified TCA Lofepramine (LOF) as first choice treatments for depression in primary care. [Peveler, R., et al. (2005). A randomised controlled trial to compare the cost-effectiveness of tricyclic antidepressants, selective serotonin reuptake inhibitors and Lofepramine]. From the below graph answer the following questions. According to the graph presented here, which is the most effective antidepressant?



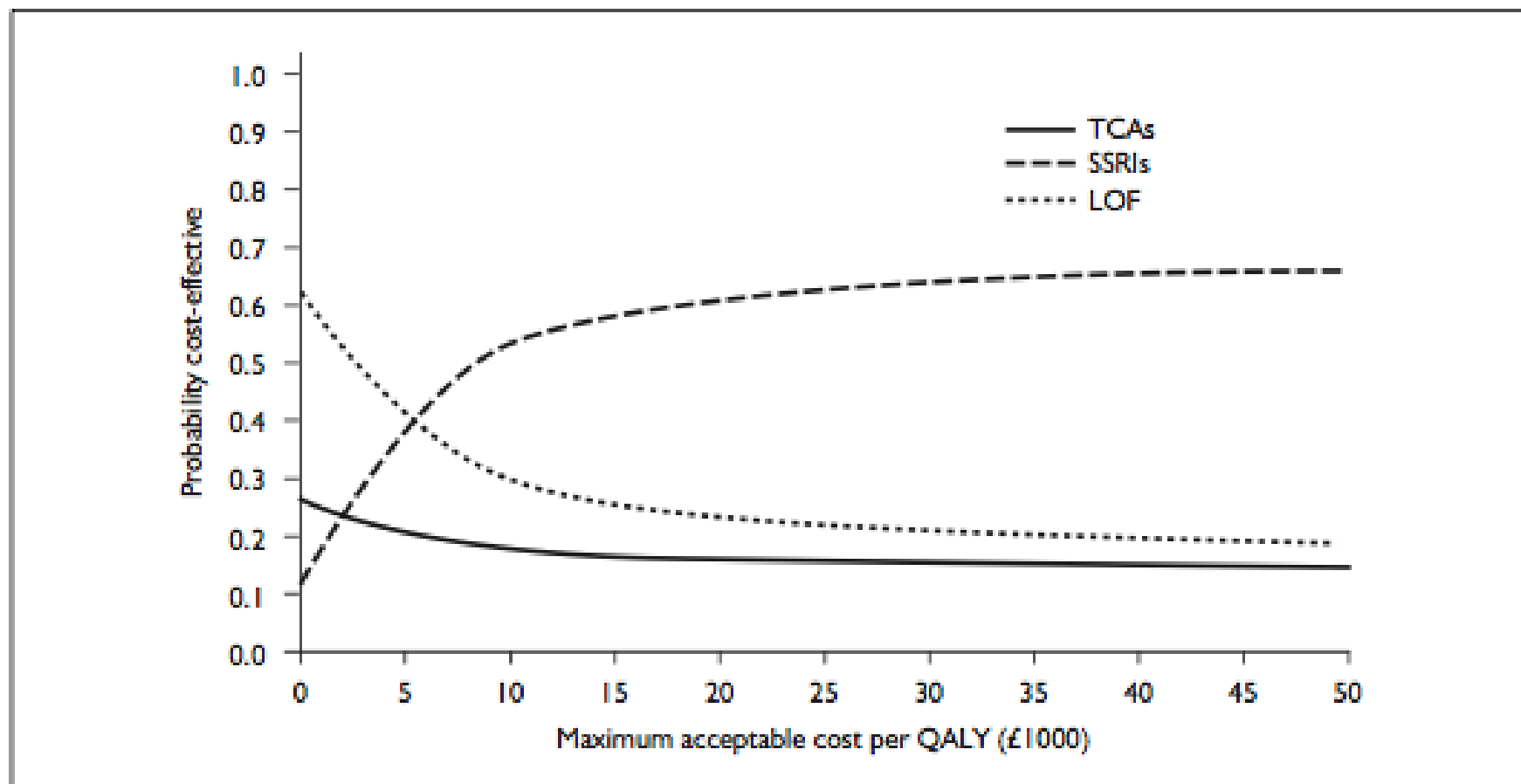


FIGURE 17 CEAC: cost per QALY

Select one:

- ☐ MAOI
- ☐ SSRI

- ☐ Lofepramine
- ☐ TCA
- ☐ None of the above

Your answer is incorrect.

Using a cost-effectiveness acceptability curve, we cannot compare the efficiency or effectiveness of various interventions; we can only infer the relative probabilities associated with acceptable cost-effectiveness for various interventions.

The correct answer is: None of the above



Question 5

Not answered

Marked out of 1

An economic analysis was conducted to determine the relative cost-effectiveness of three classes of antidepressants: tricyclic antidepressants (TCAs), selective serotonin reuptake inhibitors (SSRIs), and the modified TCA Lofepramine (LOF) as first choice treatments for depression in primary care. [Peveler, R., et al. (2005). A randomised controlled trial to compare the cost-effectiveness of tricyclic antidepressants, selective serotonin reuptake inhibitors and Lofepramine]. From the below graph answer the following questions. You are informed that the National Institute for Clinical Excellence (NICE) operates with a readiness to spend approximately £30,000 per QALY. In this case, which of the following conclusion is valid?



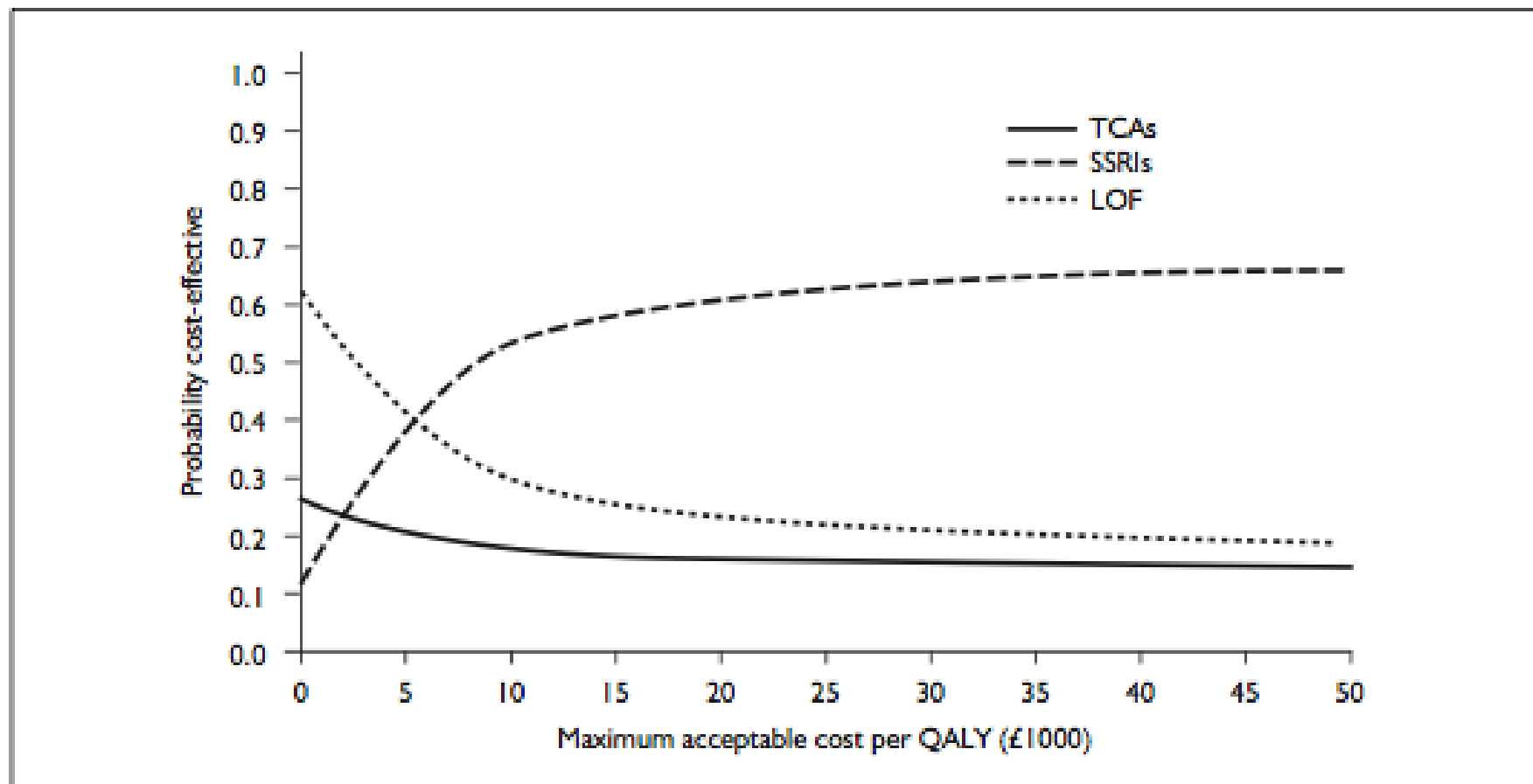


FIGURE 17 CEAC: cost per QALY

Select one:

- ☐ SSRIs are likely to be acceptably cost-effective.
- ☐ Lofepramine is likely to be acceptably cost-effective.

- ☐ A 100% probability for cost-effectiveness of SSRIs can be achieved
- ☐ There is no apparent difference in the cost-effectiveness of the three drugs
- ☐ TCAs are likely to be acceptably cost-effective.

Your answer is incorrect.

Among the 3 curves, at a level of £30,000 investment, the highest probability of being cost effective is seen for the SSRI curve. Hence SSRIs are the most likely choice for being cost-effective interventions if one can spend £30000 for a QALY gained. If the willingness to pay per QALY is considerably lower, say <£5000, other drugs may appear to be better choices (esp. lofepramine).

The correct answer is: SSRIs are likely to be acceptably cost-effective.



Question 6

Not answered

Marked out of 1

An old age psychiatry service in Liverpool compared the costs of running domiciliary clinic vs. running an outpatient clinic for all initial referrals over four years. It was concluded that over four years the cost of running a domiciliary clinic was 500,000£ while the cost for the outpatient clinic was 579,000£. What type of study is this?

Select one:

- ☐ Cost consequences
- ☐ Cost minimisation
- ☐ Cost effectiveness
- ☐ Cost benefits
- ☐ Cost utility

Your answer is incorrect.

Cost minimisation studies are used when both interventions produce same outcome; hence just the costs are compared. Outcomes are assumed to be the same and are not compared. For example consider the following: Which will cost me less- Buying CLOZARIL or buying ZAPONEX (another brand of clozapine)? Clozaril may be £3.5 per tablet vs. zaponex £3.11 per tablet.

The correct answer is: Cost minimisation

Question 7

Not answered

Marked out of 1

An RCT compared the effectiveness of using co-prescribed heroin vs. sole prescription of methadone in heroin users. An associated economic analysis concluded that there was a gain of at least 0.5 QALY per patient per year of treatment, and the cost saved was around 6000£ per patient per year of treatment. What type of study is this?

Select one:

- ☐ Cost utility
- ☐ Cost benefits
- ☐ Cost consequences
- ☐ Cost minimisation
- ☐ Cost effectiveness

Your answer is incorrect.

Cost-utility study is used when instead of clinical effect (measured using scales, questionnaires, bed occupancy etc) as .an outcome one holistically considers the disease burden, side effects and social impact etc., Here the outcome is measured in QALYs or DALYs.

The correct answer is: Cost utility

Question 8

Not answered

Marked out of 1

Which of the following is an appropriate study design to compare two different interventions with different clinical outcomes in terms of monetary outcomes?

Select one:

- ☐ Cost minimisation
- ☐ Cost effectiveness
- ☐ Cost consequences
- ☐ Cost utility
- ☐ Cost benefits

Your answer is incorrect.

When two different interventions with different clinical outcomes are compared wherein all outcomes are translated into monetary (£s) terms for comparison, then this study is called cost-benefit study. It is often used by managers to decide how to spend the allocated money effectively.

The correct answer is: Cost benefits

Question 9

Not answered

Marked out of 1

In an attempt to study the cost of adherence vs. non-adherence to treatment in depression treated at primary care centres, an investigator calculates the costs of illness in terms of the costs for hospitalization, GP-visits, specialist referral, cost of medications, and cost of each day of sick leave due to depression. Which of the following among the above can be called an indirect cost?

Select one:

- ☐ Cost of specialist referral
- ☐ Cost of medications
- ☐ Cost of hospitalisation
- ☐ Cost of GP visits
- ☐ Cost of sick absence

Your answer is incorrect.

Direct costs: Cost of hospital space, cost of drugs, equipments, salary of doctors and other professionals etc. Indirect costs: Costs due to loss of productivity for the patient, cost of having a caretaker for one's dog or a nanny for one's child when hospitalised etc. Intangible (not measurable) costs: Pain and suffering, social stigma, physical discomfort, etc.

The correct answer is: Cost of sick absence

Question 10

Not answered

Marked out of 1

615 adult patients were asked at baseline interviews and six-month follow-up interviews how much they would be prepared to spend per month for a six-month treatment that would eliminate their symptoms of depression. This measure is called

Select one:

- ☐ Per capita purchasing power
- ☐ Average cost measure
- ☐ Willingness to pay
- ☐ Incremental cost effectiveness measure
- ☐ Standard gamble measure

Your answer is incorrect.

Willingness to pay (WTP): How much people would be prepared to spend to prevent or cure a particular health state? Time trade-off measures: how many of the remaining life years would a respondent spend in trying to get cured of a particular condition (inversely - how important is it for some one to get relief from a problem). Standard gamble measures: Imagine there is a risky intervention for a health state that can either cure if successful or kill if it fails. How much risk will an individual take to get cured off such a condition with such an intervention? The measures are given as odds of success vs. odds of death - the respondents are asked to choose their preferred values.

The correct answer is: Willingness to pay

Question 11

Not answered

Marked out of 1

A Fast Track Intervention programme has been introduced by Ministry of Justice to reduce juvenile crimes. There has been no intervention available to prevent juvenile crime before this was introduced. It is estimated that when 100 students are inducted into the programme at a cost of 12,000£ per student, four juvenile criminal events are reduced. The incremental cost-effectiveness ratio for the fast track programme is given by

Select one:

- ☐ £120 per criminal event
- ☐ Insufficient information to calculate ICER
- ☐ £3,000 per criminal event
- ☐ £300,000 per criminal event
- ☐ £12,000 per criminal event

Your answer is incorrect.

The most popular economic evaluation method in health care is Cost effectiveness analysis. The results of a cost-effectiveness analysis can be summarised either as average cost-effectiveness ratio (ACER) which is equal to cost/effect or as incremental cost-effectiveness ratio (ICER), which is a measure of the additional cost per unit of health gained. $ICER = \frac{\text{Cost of proposed intervention} - \text{cost of control intervention}}{\text{Effect of proposed intervention} - \text{effect of control intervention}}$. Here, there is no control intervention. So ICER will be = Cost of proposed intervention / effect of proposed intervention. = $(12,000 \times 100) / 4 = 300,000$ per criminal event

The correct answer is: £300,000 per criminal event

Question 12

Not answered

Marked out of 1

Which of the following is not a subtype of sensitivity analyses?

Select one:

- ☐ One way analysis
- ☐ Worst case scenario measures
- ☐ Time trade off measures
- ☐ Bootstrapping measures
- ☐ Monte Carlo simulations

Your answer is incorrect.

Various assumptions are usually made in an economic analysis. For example one assumes a standard inflation, discounts for money values changes in the market etc., when calculating costs. (Note that conventionally 6% discount rates are assumed). So it is pertinent to check the effect of the outcome of such economic analysis by systematically examining what will happen if the assumptions are changed in some way. This sensitivity analysis can be done in various ways such as one-way, worst case scenario, Monte-Carlo, bootstrapping, etc. Time trade-off measure is not a sensitivity analysis; it is a proxy measure of health state preferences.

The correct answer is: Time trade off measures

Question **13**

Not answered

Marked out of 1

Dr. Hidalgo, a popular poet, comes to your clinic with significant adjustment reaction. He is worried about being a poet and not practicing medicine. He wonders how much money he has lost by not practicing medicine. Which of the following terms best suits the economic loss he might have concurred?

Select one:

- ☐ Intangible cost
- ☐ Opportunity cost
- ☐ Direct cost
- ☐ Indirect cost
- ☐ Healthcare cost

Your answer is incorrect.

Opportunity cost refers to the cost of wasted opportunities - i.e. cost of losing an opportunity to attend a business meeting, having attended a dental appointment. Every action has an opportunity cost.

The correct answer is: Opportunity cost

Question 14

Not answered

Marked out of 1

Which of the following two parameters are required for constructing a cost-effectiveness analysis curve?

Select one:

- ☐ Willingness to pay and probability of cost effectiveness
- ☐ Willingness to pay and QALYs gained
- ☐ ICER and clinical symptom rating scale scores
- ☐ ICER and QALYs gained
- ☐ Probability of cost effectiveness and QALYs gained

Your answer is incorrect.

A decision maker / NHS manager will be interested to know how sure he/she can be in probabilistic terms to make an investment. This probability can be identified from the incremental cost-effectiveness plane with reference to the decision maker's defined maximum acceptable ceiling ratio using Cost effectiveness acceptability curve - CEAC. The CEAC indicates the probability that an intervention is cost-effective compared with the alternative, given the observed data, for a range of lambda (Willingness to Pay) values. The CEAC is a method for summarizing the uncertainty in estimates of cost-effectiveness. On one side of the curve, probability of cost effectiveness is plotted; on the other side willingness to pay (for decision maker) is plotted (Retrieved from <http://www.biomedcentral.com/1472-6963/6/52>).

The correct answer is: Willingness to pay and probability of cost effectiveness

Question 15

Not answered

Marked out of 1

In a cost-effectiveness plane which of the following quadrant indicates best cost effectiveness of an intervention?

Select one:

- ☐ Southwest
- ☐ Northeast
- ☐ Southeast
- ☐ Northwest
- ☐ Central

Your answer is incorrect.

The incremental cost and incremental effect can be represented visually using the incremental cost-effectiveness plane. The horizontal axis divides the plane according to incremental cost (positive above, negative below) and the vertical axis divides the plane according to incremental effect (positive to the right, negative to the left). This divides the incremental cost-effectiveness plane into four quadrants through the origin. Each quadrant has a different implication for the decision. ICER falls in NORTHWEST: Positive costs and negative effects - no use of the intervention. ICER falls in SOUTHEAST: Negative costs and positive effects - intervention is preferred. ICER falls in NORTHEAST: Positive effects and positive costs - needs further consideration before acceptance. ICER falls in SOUTHWEST: Negative costs and negative effects - not clinically effective - needs further study (Retrieved from <http://www.biomedcentral.com/1472-6963/6/52>).

The correct answer is: Southeast

Question 16

Not answered

Marked out of 1

In a nationally funded study, respondents are asked to consider a choice between undergoing ECT and living with severe depression. The odds of relief from depression and developing amnesia when undergoing ECT are varied to see at what point the respondent considers ECT is not a worthy option. This study is best described as

Select one:

- ☐ Time trade off measurement
- ☐ Cost minimisation study
- ☐ Health preference rating study
- ☐ Standard gamble measurement
- ☐ Cost effectiveness study

Your answer is incorrect.

Standard gamble is "a method of establishing the utility of a specified health state. For chronic health states, people are asked to choose between the certainty of the specified health state for a given period of time or a gamble that involves a probability (p) of restoration to full health and a complementary probability ($1-p$) of immediate death. The value of p is changed until the respondent regards the two options as equivalent to each other. The utility of the specified health state is then given by p . A slightly modified method is needed for temporary health states or states regarded as worse than death". Shiell, A., Donaldson, C., Mitton, C., et. al (2002) Health economic evaluation. J Epidemiol Community Health, 56, 85-88.

The correct answer is: Standard gamble measurement

Question 17

Not answered

Marked out of 1

Which of the following types of economic analyses could be completed when one has access only to cost data and not outcome data?

Select one:

- ☐ Cost utility analysis
- ☐ Cost effectiveness study
- ☐ Cost benefit analysis
- ☐ Cost minimisation analysis
- ☐ Cost consequences analysis

Your answer is incorrect.

Cost minimisation analysis is a specific type of cost-effectiveness analysis in which the outcomes of the two (or more) comparators are assumed equal, thereby resulting in an assessment based solely on comparative cost. Making the assumption of equal outcomes can be risky, as such assumptions rarely hold in practice. Shiell, A., Donaldson, C., Mitton, C., et. al (2002) Health economic evaluation. J Epidemiol Community Health, 56, 85-88.

The correct answer is: Cost minimisation analysis

Question **18**

Not answered

Marked out of 1

Which of the following two parameters are required for constructing a cost-effectiveness analysis curve?

Select one:

- ☐ Willingness to pay and QALYs gained
- ☐ ICER and clinical symptom rating scale scores
- ☐ Willingness to pay and probability of cost effectiveness
- ☐ ICER and QALYs gained
- ☐ Probability of cost effectiveness and QALYs gained

Your answer is incorrect.

The cost-effectiveness acceptability curve (CEAC) illustrates the uncertainty surrounding the estimate of cost- effectiveness. A CEAC shows the probability that an intervention is cost-effective compared with the alternative, given the observed data, for a range of maximum monetary values that a decision-maker might be willing to pay for a particular unit change in outcome.

<http://bjp.rcpsych.org/content/187/2/106.full>

The correct answer is: Willingness to pay and probability of cost effectiveness

Question 19

Not answered

Marked out of 1

You review a trial which measures monetary costs and consequences of psychotherapy. What term best describes the costs of lost productivity to patients and their employers as a consequence of attending treatment?

Select one:

- ☐ Discounter cost
- ☐ Indirect cost
- ☐ Direct Cost
- ☐ Marginal cost
- ☐ Intangible cost

Your answer is incorrect.

Indirect costs or productivity losses are the labour earnings that are forgone as a result of an adverse health outcome. The decreased productivity can be a result of illness, death, side effects, or time spent receiving treatment. Indirect costs include lost earnings and productivity of both patients and the family members who take care of them. For some diseases with premature death, the indirect cost is the loss in potential wages and benefits. Indirect costs associated with premature death might be very high.

The correct answer is: Indirect cost

Question 20

Not answered

Marked out of 1

Zarkin et al., 2008 reported a cost effectiveness comparison of naltrexone and placebo in alcohol abstinence. The mean effectiveness measured as percentage days of abstinence was nearly 80% for naltrexone group while it was 73% for the placebo group. The mean cost incurred for the placebo group was \$ 400 per patient. The naltrexone group incurred a cost of \$ 680 per patient. How much additional cost needs to be spent per patient for each percentage point increase in total days of abstinence when using naltrexone compared to placebo?

Select one:

- ☐ \$ 500
- ☐ \$ 50
- ☐ \$ 7
- ☐ \$ 40
- ☐ \$ 2

Your answer is incorrect.

To calculate ICER that is asked in this question, first calculate total effectiveness gained. It is 7% extra days of abstinence. This has come at a cost of extra 280 per patient. So the $ICER = 280/7 = 40$ USD per percentage days of abstinence

The correct answer is: \$ 40

Question 21

Not answered

Marked out of 1

Cost Benefit Analysis is an economic evaluation technique that includes which of the following measurements?

Select one:

- ☐ Clinical evaluation of healthcare investments
- ☐ Costs of not undertaking the second best alternative
- ☐ Economic evaluation in terms of clinical effect
- ☐ Analysis of the benefits in terms of quality of life
- ☐ Economic evaluation in monetary terms

Your answer is incorrect.

Cost Benefit Analysis (CBA) is an economic evaluation technique that measures all the positive (beneficial) and negative (costly) consequences of an intervention or program in monetary terms. The valuation of all program outcomes in monetary units allows decision-makers to directly compare the health outcomes of different types of health interventions.

The correct answer is: Economic evaluation in monetary terms

Question 22

Not answered

Marked out of 1

An economist is interested in assessing the benefits foregone by a particular use of health resources. This can be termed as

Select one:

- ☐ Cost effect
- ☐ Inflation rate
- ☐ Direct cost
- ☐ Benefit ratio
- ☐ Implicit costs

Your answer is incorrect.

Benefits forgone by a particular use of resources is known as opportunity cost. A commonly used method to evaluate the opportunity cost of health resources is to estimate the indirect costs of the next best alternative that could have been put in place with the same resources. This is often an implicit cost not directly incurred by the health providers. BMJ. 1999 Jun 5;318(7197):1551

The correct answer is: Implicit costs

Question **23**

Not answered

Marked out of 1

In economic analysis which of the following represents the cost of pain and suffering from an intervention that cannot be quantified in monetary terms?

Select one:

- ☐ Intangible costs
- ☐ Baseline costs
- ☐ Opportunity costs
- ☐ Indirect costs
- ☐ Incremental costs



Your answer is incorrect.

Indirect cost refers to the costs of an intervention that is not directly due to the intervention itself, but can be quantified in units other than currencies e.g. number of sick days off work when undertaking a hysterectomy. The term intangible cost is used to describe the costs of pain and suffering that cannot be quantified. Opportunity cost refers to the cost of a hypothetical loss when a choice is made between two options. For example, the opportunity cost suffered by a brilliant high school student when he becomes a doctor is the money that was not earned from being a solicitor. Incremental cost refers to the additional cost of providing an intervention over and above an existing intervention.

The correct answer is: Intangible costs

Question 24

Not answered

Marked out of 1

A retrospective cohort study was used to measure inpatient service utilization in 63 consecutive treatment resistant schizophrenia patients started on clozapine from a geographical catchment area compared to a control group matched for previous inpatient service use. An intent-to-treat analysis, including those patients (n = 28) who discontinued clozapine during the study period, showed a significant reduction in number of admissions and total time spent in hospital in the 2 years following clozapine initiation compared to the previous 2 years and to the follow-up period in the control group. In terms of economic outcomes, the whole clozapine group (n = 63 includes both continuers and discontinuers) had a mean reduction of 33 bed days over the 2-year period following initiation of clozapine. In terms of local costs (£220 per adult mental health bed day, excluding capital costs), this translates into a reduction of £7341.40 in hospitalization costs per clozapine patient over the 2-year period. In those patients who continued clozapine treatment for the whole of the 2-year period (continuers), there was a two-thirds reduction in number of admissions and total time spent in hospital compared to no change in the clozapine discontinuers. [Hayhurst KP et al. The cost-effectiveness of clozapine: a controlled, population-based, mirror-image study. Journal of Psychopharmacology 16(2) (2002) 169 –175]. Whose viewpoint has been used to measure the costs?

Select one:

- ☐ Society's viewpoint
- ☐ Pharmaceutical companies' viewpoint
- ☐ Patients' viewpoint
- ☐ Service providers' viewpoint
- ☐ Carers' viewpoint

Your answer is incorrect.

The costs were calculated based on factors such as bed days and hospitalisation costs. It was not based on the patient's loss of productivity or carer's costs, or societal burden. Thus, the computation has taken the viewpoint of hospital managers.

The correct answer is: Service providers' viewpoint

The correct answer is: service providers' viewpoint



Question 25

Not answered

Marked out of 1

A retrospective cohort study was used to measure inpatient service utilization in 63 consecutive treatment resistant schizophrenia patients started on clozapine from a geographical catchment area compared to a control group matched for previous inpatient service use. An intent-to-treat analysis, including those patients (n = 28) who discontinued clozapine during the study period, showed a significant reduction in number of admissions and total time spent in hospital in the 2 years following clozapine initiation compared to the previous 2 years and to the follow-up period in the control group. In terms of economic outcomes, the whole clozapine group (n = 63 includes both continuers and discontinuers) had a mean reduction of 33 bed days over the 2-year period following initiation of clozapine. In terms of local costs (£220 per adult mental health bed day, excluding capital costs), this translates into a reduction of £7341.40 in hospitalization costs per clozapine patient over the 2-year period. In those patients who continued clozapine treatment for the whole of the 2-year period (continuers), there was a two-thirds reduction in number of admissions and total time spent in hospital compared to no change in the clozapine discontinuers. [Hayhurst KP et al. The cost-effectiveness of clozapine: a controlled, population-based, mirror-image study. Journal of Psychopharmacology 16(2) (2002) 169 –175]. Which of the following is most likely to be a weakness of this economic analysis?

Select one:

- ☐ The use of 2-years of follow-up rather than restricting follow-up within a single financial year
- ☐ The recruitment of both exposed and unexposed cohorts from the same geographical catchment
- ☐ The use of a treatment resistant sample as opposed to first episode sample
- ☐ The use of an intention-to-treat analysis in a cohort study
- ☐ The use of clinical effectiveness data obtained from a non-randomised sample

Your answer is incorrect.

When we compare effectiveness of two measures, we prefer using a RCT design. In the same way, when we compare the costs of two approaches, it is best to derive the cost data from a randomised sample. Note that economic analysis itself is NOT a study design.

The correct answer is: The use of clinical effectiveness data obtained from a non-randomised sample

The correct answer is: The use of clinical effectiveness data obtained from a non-randomised sample



Question 26

Not answered

Marked out of 1

A retrospective cohort study was used to measure inpatient service utilization in 63 consecutive treatment resistant schizophrenia patients started on clozapine from a geographical catchment area compared to a control group matched for previous inpatient service use. An intent-to-treat analysis, including those patients (n = 28) who discontinued clozapine during the study period, showed a significant reduction in number of admissions and total time spent in hospital in the 2 years following clozapine initiation compared to the previous 2 years and to the follow-up period in the control group. In terms of economic outcomes, the whole clozapine group (n = 63 includes both continuers and discontinuers) had a mean reduction of 33 bed days over the 2-year period following initiation of clozapine. In terms of local costs (£220 per adult mental health bed day, excluding capital costs), this translates into a reduction of £7341.40 in hospitalization costs per clozapine patient over the 2-year period. In those patients who continued clozapine treatment for the whole of the 2-year period (continuers), there was a two-thirds reduction in number of admissions and total time spent in hospital compared to no change in the clozapine discontinuers. [Hayhurst KP et al. The cost-effectiveness of clozapine: a controlled, population-based, mirror-image study. Journal of Psychopharmacology 16(2) (2002) 169 –175]. The average incremental effectiveness gained by clozapine users as opposed to non-clozapine users in this study is

Select one:

- ☐ 16.5 bed days per year
- ☐ 2/3rd reduction in number of admissions
- ☐ £220 per adult mental health day
- ☐ £3670.70 per year
- ☐ £231,254.10 per year

Your answer is incorrect.

Note that this question asks you what the difference in effectiveness, not the ICER. The results suggest that on average, clozapine group have 33 additional bed days 'saved' in 2 years. This translates to saving $33/2 = 16.5$ bed days per year. This value is the incremental effectiveness.

The correct answer is: 16.5 bed days per year



Question 27

Not answered

Marked out of 1

A retrospective cohort study was used to measure inpatient service utilization in 63 consecutive treatment resistant schizophrenia patients started on clozapine from a geographical catchment area compared to a control group matched for previous inpatient service use. An intent-to-treat analysis, including those patients (n = 28) who discontinued clozapine during the study period, showed a significant reduction in number of admissions and total time spent in hospital in the 2 years following clozapine initiation compared to the previous 2 years and to the follow-up period in the control group. In terms of economic outcomes, the whole clozapine group (n = 63 includes both continuers and discontinuers) had a mean reduction of 33 bed days over the 2-year period following initiation of clozapine. In terms of local costs (£220 per adult mental health bed day, excluding capital costs), this translates into a reduction of £7341.40 in hospitalization costs per clozapine patient over the 2-year period. In those patients who continued clozapine treatment for the whole of the 2-year period (continuers), there was a two-thirds reduction in number of admissions and total time spent in hospital compared to no change in the clozapine discontinuers. [Hayhurst KP et al. The cost-effectiveness of clozapine: a controlled, population-based, mirror-image study. Journal of Psychopharmacology 16(2) (2002) 169 –175]. . The total reduction in the cost of hospitalisation in clozapine users as opposed to non-clozapine users in this study is

Select one:

- ☐ £3670.70 per year
- ☐ 2/3rd reduction in number of admissions
- ☐ £220 per adult mental health day
- ☐ £3630 per year
- ☐ £231,254.10 per year

Your answer is incorrect.

This question tests the concept of ICER. ICER is given by difference in costs / difference in effectiveness. This value has already been given, for a two-year period, for each patient on clozapine (£7341.40). For 63 patients, this will translate to $63 \times £7341.40 = £462508.2$. But this is the ICER for 2 years. For one year, this will be $£462508.2/2 = £231,254.10$ per year.

The correct answer is: £231,254.10 per year



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